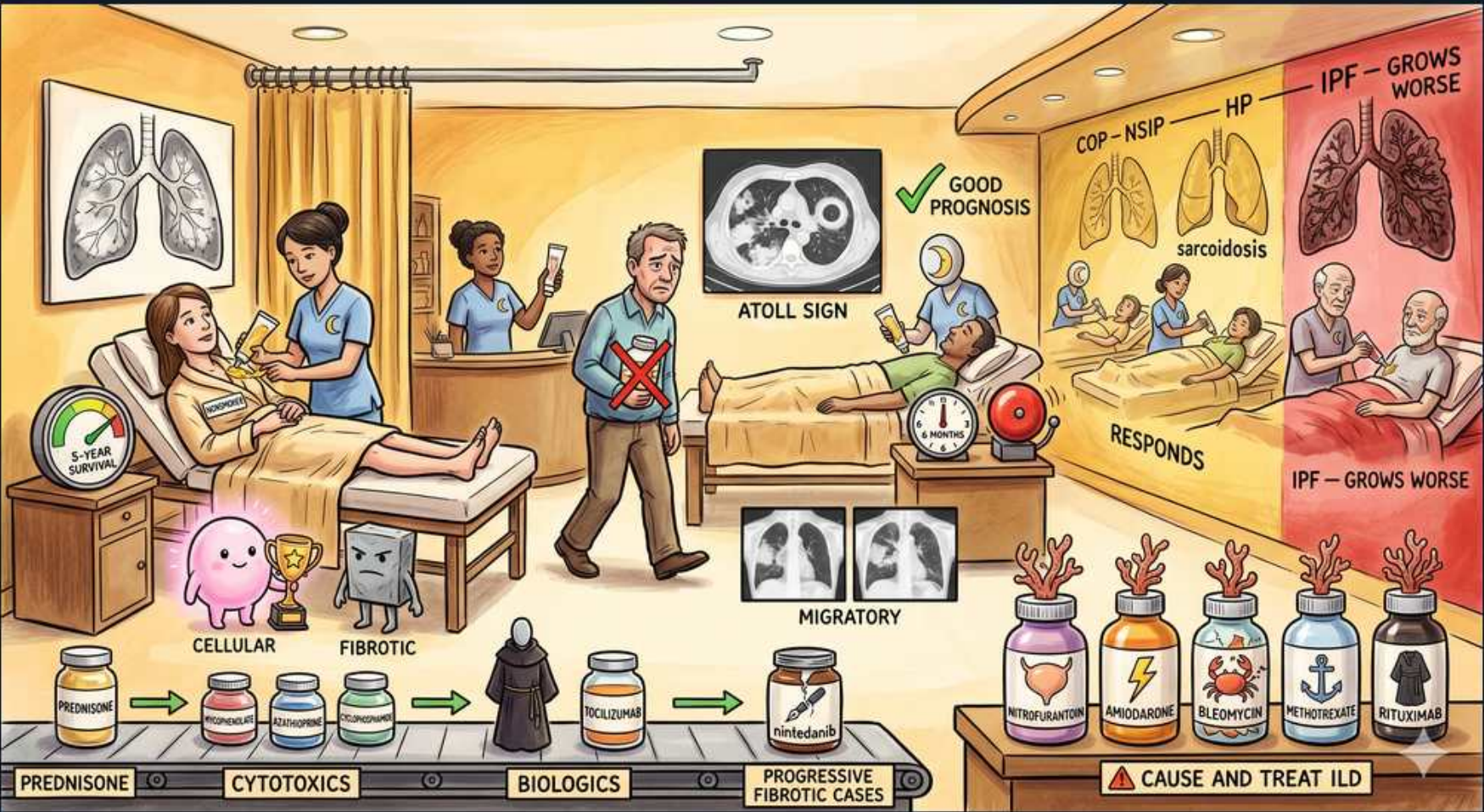


ILD — Treatment by Responsiveness & Drug-Induced Causes



1. 5-year survival gauge

WHAT YOU SEE

A dial reading '5-YEAR SURVIVAL' on the bedside table — the gauge IS the prognosis split: inflammatory ILD lives long, fibrotic UIP/IPF only ~3–5 yr.

WHAT IT ENCODES

Prognosis in ILD is dictated by the histologic/radiographic pattern: inflammatory/CELLULAR disease (COP, cellular NSIP, HP, sarcoid) is treatable and lives longer, whereas FIBROTIC UIP/IPF is progressive with a median survival of only ~3–5 years. The single most important branch point on any ILD question is 'inflammatory vs fibrotic.'

BOARD TRAP

WRONG answer: 'all ILD has a uniformly poor prognosis.' Discriminator — the pattern stratifies outcome; steroid-responsive cellular ILDs can have near-normal survival, so lumping them with IPF underestimates a treatable disease.

2. Cellular NSIP (responds)

WHAT YOU SEE

Pink soft lung figure labeled 'cellular' with a trophy

WHAT IT ENCODES

Cellular NSIP is the inflammatory, ground-glass-predominant subtype (soft pink lung) that is corticosteroid-responsive with a good prognosis, often CTD-associated. First-line is prednisone, with steroid-sparing mycophenolate or azathioprine for maintenance. The trophy = best ILD prognosis.

BOARD TRAP

WRONG answer: 'treat cellular NSIP with antifibrotics like IPF.' Discriminator — cellular = inflammatory = immunosuppression; antifibrotics are for the fibrotic/UIP end. Misclassifying a treatable inflammatory ILD as fibrotic withholds effective steroids.

3. Fibrotic (poor) figure

WHAT YOU SEE

Grey stiff lung figure labeled 'fibrotic'

WHAT IT ENCODES

Fibrotic NSIP/UIP (grey stiff lung) responds POORLY to immunosuppression and behaves like IPF — it is the progressive fibrosing phenotype where antifibrotics (nintedanib/pirfenidone) slow decline. The grey color contrasts with cellular pink to remind you fibrosis = poor steroid response.

BOARD TRAP

WRONG answer: 'a course of high-dose steroids will reverse fibrotic NSIP/UIP.' Discriminator — established fibrosis does not regress with steroids; expect at best stabilization with antifibrotics, and recall PANTHER showed immunosuppression harms IPF.

4. Prednisone (conveyor start)

WHAT YOU SEE

Bottle 'PREDNISONE' at start of treatment conveyor

WHAT IT ENCODES

Corticosteroids (prednisone) are FIRST-LINE for steroid-responsive ILDs — COP (dramatic response), cellular NSIP, sarcoidosis, and hypersensitivity pneumonitis (plus antigen avoidance). The conveyor escalates from steroids to steroid-sparing agents as needed. Mnemonic for responders: 'COP NSIP HP Sarcoid respond.'

BOARD TRAP

WRONG answer: 'give prednisone for IPF/UIP.' Discriminator — steroids do not help IPF and the PANTHER-IPF trial showed prednisone + azathioprine + NAC INCREASED death and hospitalization; the responsive vs non-responsive split is the whole point.

5. Cytotoxics step

WHAT YOU SEE

Bottle 'CYTOTOXICS' on the conveyor

WHAT IT ENCODES

Steroid-sparing immunosuppressants — azathioprine and mycophenolate mofetil (and cyclophosphamide in severe scleroderma-ILD per the Scleroderma Lung Studies) — are second-line for CTD-ILD and refractory inflammatory ILD, allowing taper of chronic steroids. They target ongoing inflammation, not fibrosis.

BOARD TRAP

WRONG answer: 'add azathioprine/mycophenolate to treat idiopathic UIP/IPF.' Discriminator — these are correct only with an inflammatory/CTD substrate; in idiopathic UIP immunosuppression is ineffective and harmful, so confirm the inflammatory pattern first.

6. Biologics step

WHAT YOU SEE

Bottle 'BIOLOGICS' / rituximab on conveyor

WHAT IT ENCODES

Biologics — chiefly RITUXIMAB (anti-CD20) — are reserved for refractory or rapidly progressive CTD-ILD (e.g., anti-synthetase/antisyndetase syndrome, severe RA-ILD, refractory scleroderma-ILD). They sit late on the escalation conveyor after steroids and conventional immunosuppressants fail.

BOARD TRAP

WRONG answer: 'use rituximab as first-line for any ILD.' Discriminator — biologics are last-line, reserved for refractory CTD-ILD; using them upfront (or in IPF) skips the evidence-based ladder and is incorrect.

7. Antifibrotic for progressive fibrosis

WHAT YOU SEE

Dark antifibrotic bottle, 'progressive fibrotic cases'

WHAT IT ENCODES

Antifibrotics (nintedanib 150 mg BID; pirfenidone titrated to 801 mg TID) are for IPF AND for any PROGRESSIVE FIBROSING ILD phenotype regardless of underlying cause (INBUILD established nintedanib for progressive non-IPF fibrotic ILD; SENSIS for scleroderma-ILD). They slow FVC decline, never reverse it.

BOARD TRAP

WRONG answer: 'escalate immunosuppression in a progressive fibrosing ILD that keeps worsening on steroids.' Discriminator — once a fibrotic ILD demonstrates progression (falling FVC despite immunosuppression), the answer shifts to ADD an antifibrotic, not more immunosuppression.

8. Atoll sign = good prognosis (COP)

WHAT YOU SEE

CT with green check 'good prognosis', 'atoll sign'

WHAT IT ENCODES

The ATOLL / reversed-halo sign (central ground-glass ringed by denser consolidation) plus a green-check good prognosis is the signature of cryptogenic organizing pneumonia, which responds rapidly and well to corticosteroids. COP often follows infection or presents as a 'pneumonia that won't clear on antibiotics.'

BOARD TRAP

WRONG answer: 'the atoll sign indicates IPF / give antifibrotics.' Discriminator — the atoll/reversed-halo sign is COP, an inflammatory steroid-responsive disease; treating it as fibrotic IPF withholds the highly effective steroids.

9. Migratory infiltrates

WHAT YOU SEE

Two side-by-side chest films labelled 'migratory' showing the opacity shifted to a new spot — the wandering shadow IS the migratory infiltrate of organizing pneumonia (COP).

WHAT IT ENCODES

MIGRATORY / shifting infiltrates (opacities that resolve in one area and appear in another over serial films) are characteristic of organizing pneumonia (COP) and also seen in chronic eosinophilic pneumonia. The wandering pattern + steroid response distinguishes it from fixed fibrotic disease.

BOARD TRAP

WRONG answer: 'persistent unchanged basal reticulation is migratory and therefore COP.' Discriminator — true migratory infiltrates move/resolve and recur; fixed, progressive basal honeycombing is UIP/IPF, an entirely different prognosis and treatment.

10. Responsive ILDs (COP/NSIP/HP/sarcoid)

WHAT YOU SEE

Pink-zone lungs 'COP, NSIP, HP, sarcoidosis — responds'

WHAT IT ENCODES

The pink 'responds' zone groups the steroid-responsive, better-prognosis ILDs: COP, cellular NSIP, hypersensitivity pneumonitis (acute/subacute — plus antigen avoidance), and sarcoidosis. These are the diseases where immunosuppression changes outcome — the opposite of the IPF red zone.

BOARD TRAP

WRONG answer: 'chronic fibrotic HP behaves like the responsive group.' Discriminator — chronic FIBROTIC HP (upper-lobe fibrosis, mosaic attenuation, air trapping) can mimic UIP and respond poorly; only the cellular/inflammatory forms reliably respond to steroids, and antigen avoidance is essential.

11. IPF grows worse

WHAT YOU SEE

Red-zone elderly figures 'IPF — grows worse'

WHAT IT ENCODES

The red 'grows worse' zone is IPF/UIP — progressive, non-responsive to immunosuppression, where antifibrotics only slow decline and transplant is the only definitive therapy. The color split (pink responds / red worsens) is the core decision tree of the whole scene.

BOARD TRAP

WRONG answer: 'IPF is steroid-responsive like the pink-zone ILDs.' Discriminator — IPF is the non-responder; the PANTHER-IPF trial showed steroids + azathioprine + NAC increased mortality, so immunosuppression is contraindicated and may harm.

12. Amiodarone (drug-induced ILD)

WHAT YOU SEE

Bottle with lightning bolt labeled amiodarone

WHAT IT ENCODES

Amiodarone causes drug-induced pneumonitis/pulmonary fibrosis (lightning bolt = the antiarrhythmic). Toxicity is dose- and duration-related; a FALLING DLCO is the most sensitive early monitor. Management is stopping the drug ± corticosteroids; CT may show high-attenuation (iodine-rich) infiltrates.

BOARD TRAP

WRONG answer: 'continue amiodarone and call the new ILD idiopathic.' Discriminator — drug-induced ILD can reverse if the agent is stopped early; always reconcile the med list before labeling ILD idiopathic, and amiodarone is the classic offender.

13. Bleomycin (drug-induced ILD)

WHAT YOU SEE

Bottle with crab/cancer icon labeled bleomycin

WHAT IT ENCODES

Bleomycin (the crab = chemotherapy for lymphoma/germ-cell tumors) causes DOSE-DEPENDENT pulmonary fibrosis, worsened by HIGH FiO2 (give the lowest tolerable supplemental oxygen perioperatively) and by older age and renal impairment. Monitor DLCO during treatment.

BOARD TRAP

WRONG answer: 'give high-flow/100% oxygen liberally to a bleomycin-exposed patient.' Discriminator — high FiO2 potentiates bleomycin lung injury; the high-yield trap is liberal perioperative oxygen, which can precipitate fatal fibrosis/ARDS.

14. Methotrexate / Nitrofurantoin / Rituximab

WHAT YOU SEE

Bottles labeled methotrexate, nitrofurantoin, rituximab in the drug row

WHAT IT ENCODES

Classic drug causes of ILD/pneumonitis: METHOTREXATE (hypersensitivity pneumonitis-like, often early, may improve with steroids and drug cessation), NITROFURANTOIN (acute hypersensitivity OR chronic fibrosis from long-term UTI prophylaxis in older women), and others. Notably some of these agents BOTH cause and treat ILD depending on context.

BOARD TRAP

WRONG answer: 'an RA patient on methotrexate with new infiltrates always has RA-ILD progression.' Discriminator — distinguish METHOTREXATE pneumonitis (drug toxicity — stop the drug) from RA-ILD itself and from opportunistic infection (PCP); the management diverges, and missing the drug cause is the trap.